



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Endocrinology

Laboratory :Endocrinology Laboratory

Sample :Serum/Plasma

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Internal Medicine-E M OPD

Req. Date : 20/03/2024

Ward : **Room No:** /

Bed:

Clinician :

Lab Sample No. : 6636

Diagnosis :

Investigation	Result	Unit	Standard Ranges
TSH	5.70	μIU/ml	0.27 - 4.2
T4	6.01	ug/dl	4.8 - 12.7
T3	0.956	ng/ml	0.8 - 2
25(OH)D3	32.6	ng/ml	11.1 - 42.9
PTH	58.1	pg/ml	15 - 65



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Biochemistry

Laboratory :Indoor Clinical Biochemistry Lab

Sample :Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Clinical Hematology & Medical Oncology-Hematology
Clinic

Req. Date : 20/03/2024

Ward : Private Ward 5th D

Room No: Room 11

Bed: Bed 11

Clinician :

Lab Sample No. : 12435

Diagnosis :

Investigation	Result	Unit	Standard Ranges
Sodium	135.2	mmol/L	135 - 145
Potassium	5.05	mmol/L	3.5 - 5
Chloride	101.3	mmol/L	90 - 107
Urea	24.3	mg/dL	10 - 50
Creatinine	1.32	mg/dL	0.5 - 1.2
Phosphorus	3.73	mg/dL	2.7 - 4.5
Calcium	9.73	mg/dl	8.8 - 10.2
Protein total	7.36	g/dL	6.4 - 8.3
Albumin	4.41	g/dL	3.4 - 4.8
Bilirubin (total)	0.25	mg/dL	0.2 - 1.2
Bilirubin (conjugated)	0.09	mg/dL	0 - 0.3
Uric Acid	6.0	mg/dl	3.4 - 7.4
AST	21.5	U/L	2 - 40
ALT	12.6	U/L	2 - 41
ALP	94	U/L	42 - 128
Mg	2.15	mg/dL	1.58 - 2.55
LDH	214	U/L	135 - 225
CRP	0.70	mg/l	0 - 5
Cholesterol	223.4	mg/dL	150 - 200
LDL chol	151.5	mg/dL	- - <130
HDL-Chol	46.0	mg/dL	35 - 55
TG	212.9	mg/dL	50 - 200



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Biochemistry

Laboratory : Indoor Clinical Biochemistry Lab

Sample : Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Clinical Hematology & Medical Oncology-Haematology
Referral

Req. Date : 19/03/2024

Ward : **Room No:** / **Bed:**

Clinician :

Lab Sample No. : 11312

Diagnosis :

Investigation	Result	Unit	Standard Ranges
CRP	0.88	mg/l	0 - 5
LDH	202	U/L	135 - 225
Sodium	137.0	mmol/L	135 - 145
Potassium	4.92	mmol/L	3.5 - 5
Chloride	103.2	mmol/L	90 - 107
Urea	26.1	mg/dL	10 - 50
Creatinine	1.13	mg/dL	0.5 - 1.2
Phosphorus	3.43	mg/dL	2.7 - 4.5
Calcium	9.87	mg/dl	8.8 - 10.2
Protein total	7.28	g/dL	6.4 - 8.3
Albumin	4.53	g/dL	3.4 - 4.8
Bilirubin (total)	0.21	mg/dL	0.2 - 1.2
Bilirubin (conjugated)	0.07	mg/dL	0 - 0.3
Uric Acid	5.8	mg/dl	3.4 - 7.4
AST	22.5	U/L	2 - 40
ALT	11.1	U/L	2 - 41
ALP	98	U/L	42 - 128
Mg	2.51	mg/dL	1.58 - 2.55



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Biochemistry

Laboratory :Indoor Clinical Biochemistry Lab

Sample :Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Clinical Hematology & Medical Oncology-Haematology
Referral

Req. Date : 21/03/2024

Ward : **Room No:** / **Bed:**

Clinician :

Lab Sample No. : 13414

Diagnosis :

Investigation	Result	Unit	Standard Ranges
Sodium	137.9	mmol/L	135 - 145
Potassium	4.89	mmol/L	3.5 - 5
Chloride	103.2	mmol/L	90 - 107
Urea	27.0	mg/dL	10 - 50
Creatinine	1.38	mg/dL	0.5 - 1.2
Phosphorus	4.01	mg/dL	2.7 - 4.5
Calcium	10.11	mg/dl	8.8 - 10.2
Protein total	7.77	g/dL	6.4 - 8.3
Albumin	4.48	g/dL	3.4 - 4.8
Bilirubin (total)	0.21	mg/dL	0.2 - 1.2
Bilirubin (conjugated)	0.07	mg/dL	0 - 0.3
AST	24.2	U/L	2 - 40
ALT	11.5	U/L	2 - 41
ALP	97	U/L	42 - 128



DEPARTMENT OF PARASITOLOGY

Postgraduate Institute of Medical Education and Research, Chandigarh

Location: S.S.Anand Block, Ground Floor (Room No. 104) Phone: 5169

Direct Microscopic examination of Stool

Name	V K Gurg	CR No.	201405136462
Age/Sex	66 Yr/M	Lab No.	P-5530
Clinical In-Charge		Received on	21/03/2024
Ward/OPD	Internal Medicine-E M OPD		

Macroscopic Examination	
Colour	Brown
Consistency	Semi Solid
Mucus	NIL
Any other remark	NIL

Microscopic Examination	
Pus Cell	NIL
RBC	NIL
Trophozoite	Nil
Cyst	Nil
Ova	Nil
Atypical Organism	Not Seen
Any other observation	NIL

Entered on 21/03/2024
Validated on 21-03-24 04:06 PM

Entered by Ranjana (Jr.Lab.Technician)
Validated by Stephen Raj(Senior Resident)



Postgraduate Institute of Medical Education and Research, Chandigarh
Department of Haematology (EMERGENCY LABORATORY)
ROUTINE URINALYSIS REPORT

Patient Name	V K Gurg	Lab ID No.	U-68
Age/Sex	66 Yr/M	Requisition Date	20/03/2024
CR Number	201405136462	Diagnosis	
Dept/Unit	Clinical Hematology & Medical Oncology-Hematology Clinic		

	REFERENCE	RESULT
COLOUR	YELLOW	-
APPEARANCE	CLEAR	-
pH	4.6 - 8	6.0
SPECIFIC GRAVITY	1.003 - 1.035	1.030
SUGAR	NEGATIVE	Negative
PROTEIN	NEGATIVE	Negative
BILRUBIN	NEGATIVE	Negative
UROBILINOGEN	0.1 - 1.0 g/dl	Negative
NITRITE	NEGATIVE	Negative
BLOOD	NEGATIVE	Negative
KETONE BODIES	NEGATIVE	Negative
ERYTHROCYTES /HPF	0 - 2	-
LEUCOCYTES/PUS CELLS /HPF	less than 5	-
EPITHELIAL CELLS /HPF	1 - 5	-
CASTS	ABSENT	-
CRYSTALS	ABSENT	-
AMORPHOUS DEPOSITS	ABSENT	-
BACTERIA	ABSENT	-
FUNGAL SPORES	ABSENT	-
TYPE of CAST/CRYSTAL/ANY OTHER ORGANISMS/COMMENTS	wbc -ve	

1. The chemical tests are performed by dipstick methods and values are approximate.
2. False positive and negative results are described due to various reasons in dipstick method.
3. All abnormal results need to be correlated with patients clinical condition.
4. An abnormal result may not always indicate a diseased state as they could be due to improper sampling, storage and transport.
5. The presence of pus cells and bacteria in the presence of many epithelial cells could be due to improper sample collection especially in females.
6. The presence of fungal spores or bacteria in the absence of pus cells could be due to contamination.
7. The exact characterization of crystals needs proper clinical history especially that of drugs and further confirmation needs chemical analysis.
8. Positivity for blood could be due to hematuria, hemoglobinuria as well as myoglobinuria.

Validated by

-(-)

Validated on

20-03-24 08:03 PM



Postgraduate Institute of Medical Education and Research, Chandigarh

Department of Haematology (EMERGENCY LABORATORY)

HAEMOGRAM REPORT

Patient Name V K Gurg Lab ID No. H612
Age/Sex 66 Yr/M Requisition date 19/03/2024
CR number 201405136462 Diagnosis
Dept./Unit Clinical Hematology & Medical Oncology-Haematology Referral

	Result	Reference Range
Haemoglobin, Hb (gm/dL)	8.8	13.5 - 17.5
Haematocrit / PCV (%)	30.2	41 - 53
Red cell count, RBC ($\times 10^{12}$ /L)	3.80	4.5 - 5.9
Mean Corpuscular Volume, MCV (fl)	79.5	80 - 100
Mean Corpuscular Haemoglobin, MCH (pg)	23.2	26 - 34
Mean Corpuscular Hb Conc., MCHC (gm/dL)	29.1	31 - 37
Red Cell Distribution Width, RDW-CV (%)	16.6	11.0 - 14.5
Reticulocyte Count (%)	-	1.0 - 2.7
ESR (mm in 1 st hour)	-	0 - 13
Platelet Count ($\times 10^9$ /L)	183	150 - 450
Total Leucocyte Count, TLC ($\times 10^9$ /L)	6.45	4.0 - 11.0

Differential Leucocyte Count					
	Result	Reference Range		Result	Reference Range
Neutrophils (%)	71.8	41 - 72	Absolute Neutrophil Count ($\times 10^9$ /L)	4.63	1.8 - 7.7
Lymphocytes (%)	16.3	15 - 45	Absolute Lymphocyte Count ($\times 10^9$ /L)	1.05	1 - 4.8
Eosinophils (%)	1.4	0 - 6	Absolute Eosinophil Count ($\times 10^9$ /L)	0.09	0.1 - 1
Monocytes (%)	10.2	2 - 10	Absolute Monocyte Count ($\times 10^9$ /L)	0.66	0.2 - 1
Basophils (%)	0.3	0 - 1	Absolute Basophil Count ($\times 10^9$ /L)	0.02	0 - 0.2
Blasts (%)	-				
Promyelocytes (%)	-				
Myelocytes (%)	-				
Metamyelocytes (%)	-				
nRBC (per 100 WBC)	-				

Peripheral Smear / Comments

Note: This report is being released without smear review. Results should be interpreted only by authorized personnel. Please contact the lab, if required.

Advice

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Validated On

19-03-24 02:53 PM

Validated By

Sonal Seth(Senior Resident)



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Virology

Laboratory :Viral Serology Lab

Sample :Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Internal Medicine-E M OPD

Req. Date : 20/03/2024

Ward : **Room No: /**

Bed:

Clinician :

Test : EBV VCA IgM Ab (serology)

Lab Sample No. : 173

Diagnosis :

Investigation	Result	Unit	Standard Ranges
EBV VCA IgM antibody by micro ELISA test	Negative	--	--
Advice	-	--	--



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Gastroenterology

Laboratory :Microbiology

Sample :Stool

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Clinical Hematology & Medical Oncology-Hematology
Clinic

Req. Date : 22/03/2024

Ward : Private Ward 5th D

Room No: Room 11

Bed: Bed 11

Clinician :

Test : C-Difficle Toxin

Lab Sample No. : 8127

Diagnosis :

C-Difficle toxin Non-Reactive

Sample OD 0.04

Normal Cutoff value ≥ 0.08

Remarks -



Postgraduate Institute of Medical Education and Research, Chandigarh

Department of Haematology (EMERGENCY LABORATORY)

HAEMOGRAM REPORT

Patient Name V K Gurg Lab ID No. H794
Age/Sex 66 Yr/M Requisition date 20/03/2024
CR number 201405136462 Diagnosis
Dept./Unit Private Ward 5th D

	Result	Reference Range
Haemoglobin, Hb (gm/dL)	9.0	13.5 - 17.5
Haematocrit / PCV (%)	29.0	41 - 53
Red cell count, RBC ($\times 10^{12}/L$)	3.87	4.5 - 5.9
Mean Corpuscular Volume, MCV (fl)	74.9	80 - 100
Mean Corpuscular Haemoglobin, MCH (pg)	23.2	26 - 34
Mean Corpuscular Hb Conc., MCHC (gm/dL)	30.9	31 - 37
Red Cell Distribution Width, RDW-CV (%)	17.6	11.0 - 14.5
Reticulocyte Count (%)	-	1.0 - 2.7
ESR (mm in 1 st hour)	-	0 - 13
Platelet Count ($\times 10^9/L$)	160	150 - 450
Total Leucocyte Count, TLC ($\times 10^9/L$)	6.7	4.0 - 11.0

Differential Leucocyte Count					
	Result	Reference Range		Result	Reference Range
Neutrophils (%)	66.6	41 - 72	Absolute Neutrophil Count ($\times 10^9/L$)	4.5	1.8 - 7.7
Lymphocytes (%)	20.9	15 - 45	Absolute Lymphocyte Count ($\times 10^9/L$)	1.4	1 - 4.8
Eosinophils (%)	2.1	0 - 6	Absolute Eosinophil Count ($\times 10^9/L$)	0.1	0.1 - 1
Monocytes (%)	9.9	2 - 10	Absolute Monocyte Count ($\times 10^9/L$)	0.7	0.2 - 1
Basophils (%)	0.5	0 - 1	Absolute Basophil Count ($\times 10^9/L$)	0.0	0 - 0.2
Blasts (%)	-				
Promyelocytes (%)	-				
Myelocytes (%)	-				
Metamyelocytes (%)	-				
nRBC (per 100 WBC)	0.1				

Peripheral Smear / Comments

Note: This report is being released without smear review. Results should be interpreted only by authorized personnel. Please contact the lab, if required.

Advice

-

Validated On

20-03-24 01:40 PM

Validated By

Aamir Khan(Jr.Lab.Technician)



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Endocrinology

Laboratory :Endocrinology Laboratory

Sample :Serum/Plasma

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Internal Medicine-E M OPD

Req. Date : 20/03/2024

Ward : **Room No:** /

Bed:

Clinician :

Test : HbA1c

Lab Sample No. : 6636

Diagnosis :

Investigation	Result	Unit	Standard Ranges
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HbA1c	5.8	%	3.8 - 5.9
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Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Biochemistry

Laboratory :Biochemistry Special Test Lab

Sample :Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Internal Medicine-E M OPD

Req. Date : 20/03/2024

Ward : **Room No:** /

Bed:

Clinician :

Test : Ferritin

Lab Sample No. : 359

Diagnosis :

Investigation	Result	Unit	Standard Ranges
Ferritin	17.95	ng/ml	30 - 400



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Haematology

Laboratory :Coagulation Lab

Sample :Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Clinical Hematology & Medical Oncology-Hematology
Clinic

Req. Date : 20/03/2024

Ward : Private Ward 5th D

Room No: Room 11

Bed: Bed 11

Clinician :

Lab Sample No. : G285

Diagnosis :

Investigation	Result	Unit	Standard Ranges
PT	10.5	sec	10.0 - 13.0
PTI	100	%	80 - 100
INR	0.89	-	* - *
PTTK/APTT	26.2	sec	27.0 - 36.0
Fibrinogen Assay	4.12	g/L	2.0 - 4.8
D dimer Assay	239	nanogm/ml	0 - 240



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Virology

Laboratory : Viral Serology Lab

Sample : Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Internal Medicine-E M OPD

Req. Date : 20/03/2024

Ward : **Room No:** /

Bed:

Clinician :

Test : HCV Ab (anti HCV)

Lab Sample No. : 8403

Diagnosis :

Method : CHEMILUMINES CENCE

HCV antibody : Non-Reactive

Advice : -



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Haematology

Laboratory : Hemolytic Anemia Lab

Sample : Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Clinical Hematology & Medical Oncology-Haematology
Referral

Req. Date : 22/03/2024

Ward : **Room No:** / **Bed:**

Clinician :

Lab Sample No. : 56

Diagnosis :

Serum Iron **22.7** 65 - 175µg/dl

Unsaturated Iron Binding Capacity 349.8 110 - 370µg/dL

Total iron binding capacity 372.50 228 - 428µg/dl

Percentage Saturation **6.09** 20 - 40%

Remarks -

Serum Ferritin **17.85** 30 - 400ng/ml

IMPRESSION Consistent with iron deficiency. Please correlate clinically.

Advice -



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Virology

Laboratory :Viral Serology Lab

Sample :Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Internal Medicine-E M OPD

Req. Date : 20/03/2024

Ward : **Room No: /**

Bed:

Clinician :

Test : EBV VCA IgM Ab (serology)

Lab Sample No. : 173

Diagnosis :

Investigation	Result	Unit	Standard Ranges
EBV VCA IgM antibody by micro ELISA test	Negative	--	--
Advice	-	--	--



Postgraduate Institute of Medical Education and Research, Chandigarh

Department of Medical Parasitology

Location: S.S.Anand Block, Ground Floor, Room No. 107, Phone: 0172-2755124

Patient Name	Age/Sex	C R No	Ward/OPD	Lab No.	Bed No.
V K Gurg	66 Yr/M	201405136462	Clinical Hematology & Medical Oncology- Haematology Referral	T-1270	
Clinical Diagnosis		Previous Result, if any		Antiparasitic Therapy	
		-		-	
Specimen		Collection Date and Time		Consultant	
Clotted Blood (Serum)		20/03/2024			
Antibody Detection(IgG) for Toxoplasmosis By ELISA		Negative			
Remarks		-			
Antibody Detection (IgM) for Toxoplasmosis By ELISA		Negative			
Remarks		-			

This is computer generated report.No signature Required

Validated by SR/KG/RS



Postgraduate Institute of Medical Education & Research
Chandigarh

Department of Virology

Laboratory :Viral Serology Lab

Sample :Blood

CR No : 201405136462

Patient Name : V K Gurg

Age/Sex : 66 Yr/M

Dept-Unit : Internal Medicine-E M OPD

Req. Date : 20/03/2024

Ward : **Room No: /**

Bed:

Clinician :

Test : HSV IgM 1+2 Antibody (serology)

Lab Sample No. : 177

Diagnosis :

Investigation	Result	Unit	Standard Ranges
HSV (1+2) IgM antibody by capture ELISA test	Negative	--	--
Advice	-	--	--



Name : Mr. VIPAN KUMAR GARG

Lab No. : 460601933

Ref By : Dr. PGIMER CHANDIGARH

Collected : 21/3/2024 3:18:00PM

A/c Status : P

Collected at : FPSC-YOMED INDIA

SCO-30,SECTOR 11 D,NEAR POST OFFICE

SECTOR 11,SECTOR- 11 D,

CHANDIGARH,

CHANDIGARH160011

CHD ,IND

Age : 66 Years

Gender : Male

Reported : 23/3/2024 5:41:55PM

Report Status : Final

Processed at : LPL-NATIONAL REFERENCE LAB

National Reference laboratory, Block E,

Sector 18, Rohini, New Delhi -110085

Test Report

Test Name	Results	Units	Bio. Ref. Interval
CYTOMEGALOVIRUS (CMV) ANTIBODY, IgG, SERUM (CLIA)	154.00	U/mL	<12.00

Interpretation

RESULT IN U/mL	REMARKS
<12.00	Negative
12.00-14.00	Equivocal
>=14.00	Positive

Note

1. This assay is used for quantitative detection of specific IgG antibodies to *Cytomegalovirus* (CMV) in serum samples.
2. Positive result for CMV IgG indicates past infection with *Cytomegalovirus*. Pregnant females with positive CMV specific IgG antibodies are considered to be immune and hence risk of transmission of infection to fetus is minimal.
3. Equivocal results should be re-tested in 10-14 days.
4. Negative result indicates person has not been exposed to CMV infection in the past. Pregnant females with negative CMV specific IgG antibodies are considered at risk of transmission of infection to fetus. Patients with negative results in suspected disease should be re-tested after 10-14 days. False negative results can be due to immunosuppression or due to low/undetectable level of IgG antibodies.
5. To differentiate between recent and past infection, CMV IgG avidity test is indicated.
6. The result should be interpreted in conjunction with clinical finding and other diagnostic tests.
7. The magnitude of the measured result is not indicative of the amount of antibody present.

Comments

Cytomegalovirus (CMV) is a member of the Herpesviridae family of viruses and usually causes asymptomatic infection after which it remains latent in patients, primarily within bone marrow derived cells. Seroprevalance increases with age. Infections can be acquired through direct contact with individuals shedding the virus. CMV can be transmitted vertically and horizontally, and infection can be classified as being acquired before birth (pre-natal), at the time of birth (perinatal) or later in life (postnatal). Infections are usually mild and asymptomatic but may pose a significant medical risk in pregnant women, newborns and immunocompromised individuals. In utero infection can lead to varying degrees of mental retardation, chorioretinitis, hearing loss and neurologic problems. Single test results are useful in screening organ transplant recipients and donors before transplantation and donors of blood products that are to be





Name : Mr. VIPAN KUMAR GARG

Lab No. : 460601933

Ref By : Dr. PGIMER CHANDIGARH

Collected : 21/3/2024 3:18:00PM

A/c Status : P

Collected at : FPSC-YOMED INDIA

SCO-30,SECTOR 11 D,NEAR POST OFFICE

SECTOR 11,SECTOR- 11 D,

CHANDIGARH,

CHANDIGARH160011

CHD ,IND

Age : 66 Years

Gender : Male

Reported : 23/3/2024 5:41:55PM

Report Status : Final

Processed at : LPL-NATIONAL REFERENCE LAB

National Reference laboratory, Block E,

Sector 18, Rohini, New Delhi -110085

Test Report

Test Name	Results	Units	Bio. Ref. Interval
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administered to premature infants and bone marrow transplant patients. In patients with CMV neurologic disease, CSF may be tested for presence of CMV specific antibodies. However in interpreting CSF IgG CMV levels it should be done in conjunction with serum IgG levels and ratio of 4:1 is considered significant indicating neurological CMV infection.

EPSTEIN-BARR VIRUS ANTIBODY TO VIRAL CAPSID ANTIGEN (VCA), IgG, SERUM (EIA)	30.53	Index	<9
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Interpretation

Index	REMARKS
<9	Negative
9-11	Equivocal
≥11	Positive

Note:

Equivocal results should be retested after 2-4 weeks.

Comments:

Infectious mononucleosis is an acute lymphoproliferative disease common in children and young adults caused by Epstein Barr Virus (EBV) which is one of the Herpes viruses. VCA IgG antibodies occur early in the disease, reach highest titres within 2-4 weeks and persist for many years, maybe lifelong in some individuals.





Name : Mr. VIPAN KUMAR GARG
Lab No. : 460601933
Ref By : Dr. PGIMER CHANDIGARH
Collected : 21/3/2024 3:18:00PM
A/c Status : P
Collected at : FPSC-YOMED INDIA
SCO-30,SECTOR 11 D,NEAR POST OFFICE
SECTOR 11,SECTOR- 11 D,
CHANDIGARH,
CHANDIGARH160011
CHD ,IND

Age : 66 Years
Gender : Male
Reported : 23/3/2024 5:41:55PM
Report Status : Final
Processed at : LPL-NATIONAL REFERENCE LAB
National Reference laboratory, Block E,
Sector 18, Rohini, New Delhi -110085

Test Report

Test Name	Results	Units	Bio. Ref. Interval
HERPES SIMPLEX VIRUS (HSV) 1 + 2 IgG, SERUM (CLIA)	<0.500	Index	<0.90

Interpretation

RESULT (INDEX)	REMARKS
<0.90	Negative
0.90-<1.10	Equivocal
>=1.10	Positive

Note

1. This assay is used for qualitative detection of specific IgG antibodies to Herpes Simplex virus (1+2) in serum samples only. **However in interpreting CSF HSV IgG levels it should be done in conjunction with serum IgG levels and ratio of 4:1 is considered significant indicating HSV encephalitis.**
2. Positive result indicates past infection with Herpes Simplex virus or administration of HSV immunoglobulins. Pregnant females with positive HSV specific IgG antibodies are considered to be immune and hence risk of transmission of infection to fetus is minimal.
3. Equivocal results should be re-tested in 10-14 days.
4. Negative result indicates person has not been exposed to Herpes Simplex virus in the past. Patients with negative results in suspected disease should be re-tested after 10-14 days. False negative results can be due to immunosuppression or due to low/undetectable level of IgG antibodies.
5. HSV serology cannot distinguish genital from nongenital infections.
6. The result should be interpreted in conjunction with clinical finding and other diagnostic tests.
7. The magnitude of the measured result is not indicative of the amount of antibody present.

Comments

Herpes simplex virus (HSV) types 1 and 2 are members of the Herpesviridae family, and produce infections that may range from mild stomatitis to disseminated and fatal disease. Infections with HSV types 1 and 2 can differ significantly in their clinical manifestations and severity which can range from gingivostomatitis, keratitis, encephalitis, vesicular skin eruptions, aseptic meningitis, neonatal herpes, genital tract infections, and disseminated primary infection.

HSV type 2 primarily causes urogenital infections and is found almost exclusively in adults. HSV type 1 is closely associated with orolabial infection, although genital infection with this virus can be common in certain populations. Once infection occurs, HSV persists in a latent state in sensory ganglia from where it may





Name : Mr. VIPAN KUMAR GARG
Lab No. : 460601933
Ref By : Dr. PGIMER CHANDIGARH
Collected : 21/3/2024 3:18:00PM
A/c Status : P
Collected at : FPSC-YOMED INDIA
SCO-30,SECTOR 11 D,NEAR POST OFFICE
SECTOR 11,SECTOR- 11 D,
CHANDIGARH,
CHANDIGARH160011
CHD ,IND

Age : 66 Years
Gender : Male
Reported : 23/3/2024 5:41:55PM
Report Status : Final
Processed at : LPL-NATIONAL REFERENCE LAB
National Reference laboratory, Block E,
Sector 18, Rohini, New Delhi -110085

Test Report

Test Name	Results	Units	Bio. Ref. Interval
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re-emerge to cause periodic recurrence of infection induced by many stimuli, which may or may not result in clinical lesions.

Asymptomatic infections may occur in healthy individuals and during pregnancy. In immunocompromised patients the disease is more severe and they are more likely to have frequent HSV recurrences. This suggests that serum antibody and virus-specific cell-mediated immunity contribute to recovery. Pregnant women who develop genital herpes are two to three times more likely to have spontaneous abortions or deliver a premature infant than are pregnant non-infected women. Infection in neonates occur during passage through birth canal and may result in neurological damage.

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-----End of report -----



IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory. •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician. •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor. •The report does not need physical signature.

(#) Sample drawn from outside source.

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.

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National Reference lab, Delhi, a CAP (7171001) Accredited, ISO 9001:2015 (FS60411) & ISO 27001:2013 (616691) Certified laboratory.

